

CHAPTER 5

IFS and the Body

Parts can affect the body and, conversely, IFS can affect somatic processes, including diseases that are not ordinarily considered psychosomatic. My (RS) early collaboration with Ron Kurtz (who developed the body-centered Hakomi method of psychotherapy; Kurtz, 1990) taught me that most clients are able to locate parts in the body and follow physical sensations to the inner world, where they can make deep connections. In this chapter, we describe how IFS approaches embodiment, which includes a number of ideas developed by Susan McConnell (2013), a Hakomi therapist who was one of the first lead trainers for the educational arm of IFS (the IFS Institute). Susan has used her expertise in bodywork and somatic psychology to broaden the IFS model.

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THE SELF OUT OF THE BODY

The body is the vessel for parts and the Self. Therefore, when we speak of a “person” in IFS, we mean the body, the parts, and the Self. When life experiences inject burdens into this system, parts may decide to separate the Self from the body, which, in turn, increases the risk of the system absorbing more burdens. For example, when we face overwhelming danger, a protective part will often act as a bodyguard, “taking the hit” (as these parts report), while other parts “push” the Self out of the body. This description jibes with frequent reports from clients that they watched from the ceiling as they were abused. Although removing the Self from the body seems to be a useful survival skill in terrifying moments, it also leaves a blanketing inner numbness, which, though relieving, is deadening.

In contrast, when parts unburden, we feel warmer, more grounded, and more alert as well as clearer, calmer, and more acutely aware mentally. Since the Self is miraculously resilient and

immune from damage, we may wonder why parts try to protect it. In some cases young parts may not be aware that the Self doesn't need protection, but in many cases perpetrators punish children for exhibiting qualities of the Self (e.g., curiosity, confidence, connectedness, courage, compassion). In response to this kind of targeted abuse, parts push the Self out of the body to prevent more punishment.

Although the Self can be pushed out of the body completely, it can also be disembodied along a continuum. Most of us have had experiences that caused our Selves to disembody to some extent at some point. During 4 years of college football, I (RS) charged full speed into guys almost twice my size many times and suffered, among other injuries, several concussions. Afterward I needed years of inner work to get back to feeling certain sensations and emotions in my body. During the course of that inner work, I discovered that my father's rage attacks and spankings had scared the Self out of me, leaving me desensitized. In the grip of my own rageful protector, I wanted to knock people down and was able to disregard physical pain even while my saner parts were screaming at me to stop. When burdens take us out of our bodies, we are at much higher risk of engaging in physically dangerous activities that cause damage and lead to further disembodiment.

EXPERIENCING THE SELF IN THE BODY

Only our parts can decide to allow the Self back into the body. But there are many ways to embody once reactive protectors give their permission. For example, we can use the breath or focus on the heart, bones, spine, and crown of the head. Sometimes movement helps. The practice of yoga, meditation, martial arts, and other mind-body healing modalities can all help parts feel welcome and safe so the Self can embody. With these strategies we can access, strengthen, and stabilize the Self in the body, which brings alignment, groundedness, fluidity, spaciousness, and often tingling. When the Self is embodied, we are sensate and open to change, and the breath is full and effortless (McConnell, 2013). There is also plenty of space for our parts.

Embodiment is important for Self-leadership for a couple of reasons. First, the Self has access to our physical hardware and, second, with a solid sense of the Self in the body, parts trust its leadership more. Ironically, experiencing the Self in this grounded, physical way can also generate a transcendent expansiveness that goes beyond the boundaries of individuality and the body to the feeling of oceanic unity. We do, however, have one important caveat about practices that are designed to speed embodiment.

Embodying the Self can terrify people who have suffered severe abuse. As mentioned above, savvy perpetrators try to prevent the victim from having access to the Self. To keep a victim compliant and brainwashed, such a perpetrator will increase punishment whenever the child (or adult) embodies Self-energy (any of those *C*-word qualities ranging from curiosity to compassion). As a result, survivors of severe abuse risk backlash from their protectors when they try to embody the Self. This backlash might include suicidal thoughts or urges, physical illness, pain, or fear of the therapist and the urge to quit therapy. Thus we make a point of asking the client to check if any protectors are afraid to let her embody before guiding her to do so directly. If any parts are afraid, we address their fears before proceeding.

I (RS) have developed a daily embodiment practice that helps me find activated parts and discern my level of Self-embodiment. Throughout the day I check my heart frequently to see how open it is, because when I'm embodied it's quite open. I also check to see if a vibrating energy is running through my heart. We call this Self-energy; Eastern spiritual traditions call it *qi* or *chi* or *prana*. If my heart is blocked, I know that a protector is doing the blocking. I might ask it to relax, or I might focus on helping it trust that I don't need that kind of protection anymore. I also know that my most active protectors hang out as tension above my eyebrows, weight on my shoulders, and clutching in my throat, and I'll visit those places to reassure those parts. As they release, I immediately notice more spaciousness and calm in body and mind.

PARTS IN THE BODY, AND THE BODY AS A TOOL FOR PARTS

Parts not only embody or disembody, they can be more or less embodied. The more embodied they are, the more their experiences and approach to life is illustrated in posture, gesture, voice, and facial expressions. They can, for example, show up as muscle tension, pressure, or trouble breathing; they can be numb, weak, chronically hot or cold; they can be starved or want the insulation of fat to avoid attracting sexual interest; they can also be sexually overstimulated. In 2013 Susan McConnell wrote:

As parts absorb burdens over the course of a lifetime, the body's awareness, breathing patterns, ability to resonate with others, to move with ease, grace, and freedom, and to give and receive touch are all adversely affected, [but] psychic as well as physical injury [that] occurs in the body ... can be healed in the body. (pp. 105–106)

I (RS) had one client who found a manager creating chronic fatigue symptoms in order to get care and rest for a needy little girl part. Another client who was in a cult discovered a part who was

making her sick with environmental illness with the aim of getting the cult leaders to reject her. Her illness was, at the same time, an eloquent way for her parts to communicate their overwhelming sense that her environment was toxic. Personally, I am aware of a part who has such high expectations of my body that it feels betrayed by illness and aging. Some of my parts relate to my body as if it were a bothersome plant, or a pet that must be kept in shape, nourished, and dragged around. I have had clients whose parts hated and feared the body. These parts (and they are not uncommon) can cause us to neglect, judge, numb, or immobilize the body. They can also use powerful sensations, energies, and hungers in the body to pursue various goals. They are capable of depleting, wearing out, or even killing the body.

Parts use the body for their own purposes, which tend to be different for exiles than for protectors. Exiles use it to signal their need for help; managers use it to exert control; and firefighters use the body to either distract from emotional pain or from overinhibition (the work of managers). In the service of these goals, parts can distract from and also cause all manner of physical dysfunction and pain. Firefighters can wreak havoc in the body with behaviors such as addiction, eating disorders, promiscuity, and self-harm, though physical damage is often a side effect rather than the goal. In addition, when protectors are polarized the body can become a battleground. For example, while one part numbs, another may fight to amplify physical sensations.

When we look at protective roles, we can more or less match them to physical symptoms. Managers—who need to contain, suppress, hold, freeze, and control—tend to show up in the muscles and fascia. Although they can be found anywhere in the body, managers can more easily handle the energies at junctions: all joints, the pelvic and respiratory diaphragms, the throat and jaw, the shoulders, and the lower back. Firefighters tend to activate the endocrine and nervous systems in the service of fight and flight, releasing stress hormones, increasing heart and breath rates, and dilating the pupils. As well, firefighters use physical arousal or desire (e.g., cravings for food, sex, alcohol, drugs, or sleep) to distract from emotions they view as a threat. Meanwhile, exiles often hide out in or around the heart, gut, and back.

We are by no means implying that diseases are solely caused or maintained by parts. Of course genetics, viruses, bacteria, injury, and environmental toxins all affect the body and can make us sick regardless of the state of our psyche. But we are saying that when parts want to deliberately affect biological processes, they seem capable of doing so. We have heard from parts who claim to have increased immune responses (causing autoimmune disorders) or lowered immune responses (allowing viruses or bacteria to flourish). We have also heard from parts who claim that they took advantage of existing organ weaknesses or genetic predispositions to create certain symptoms in the

service of some agenda. We have no clue how they could have done this, but we do know that our clients got better when parts who said they were doing something to the body agreed to stop.

Parts can also make medical conditions worse unintentionally. When they try to fight or distract from an autoimmune disorder such as rheumatoid arthritis, the resulting damage can be equal to or greater than the original problem. And when they fail to share information about the body with medical providers, or forget to take medication and attend appointments, they put us at risk (Livingston & Gaffney, 2013). IFS has also been used to form alliances with the rebellious and denying parts in adolescents who have Type 1 diabetes, a particularly vulnerable population. For these kids medical noncompliance, which can include some predictable behaviors such as eating forbidden foods and forgetting to take insulin, can lead to amputations or even death.

IFS AND MEDICINE

Nancy Shadick, Nancy Sowell, and their colleagues (2013) conducted a study in which IFS therapy was offered to patients with rheumatoid arthritis (RA). Thirty-seven patients who had RA received 9 months of group and individual IFS therapy and were compared to a control group of 40 patients with RA who received just an educational intervention. Both groups were followed for 9 months after completion. The IFS treatment group showed improvement in overall pain and physical function, as well as in self-assessed joint pain, self-compassion, and depressive symptoms, all of which were sustained at follow-up. In addition, I (RS) have been using IFS successfully with patients on a wide variety of medical problems for 20 years. I've worked with patients who had cancer, lupus, or pain of all varieties, among other illnesses. If parts were involved in the creation or maintenance of the illness or its symptoms, the patient improved, often to the point of remission.

THE ACE STUDIES

In 1995 Kaiser Permanente HMO in California and the Centers for Disease Control and Prevention jointly initiated the Adverse Childhood Experiences (ACE) study, a path-breaking epidemiological survey that demonstrated an astonishing correlation between childhood maltreatment, later-life medical illness, and premature death (Corso, Edwards, Fang, & Mercy, 2008; Wylie, 2010). While getting physical exams, over 17,000 patients were interviewed for the ACE study regarding childhood experiences of abuse, neglect, and household dysfunction. Researchers examined these data for correlations between adverse childhood events and adult emotional and physical health. Not surprisingly, people with high ACE scores also engaged in many high-cost behaviors such as

smoking, drinking, overeating, and drug abuse, which we categorize as firefighter activities. These, of course, increased their risk for physical illness. However, the study also found that even after taking risk activities into account, a strong correlation between adverse childhood events and diseases such as cancer, coronary artery disease, and chronic pulmonary disease remained. Although these findings surprised the researchers, they validated what we in the field of mental health have long intuited about trauma and the body. The findings also suggest that the reverse might be true: If a wounded psyche can injure the body a restored psyche might help to heal the body. (For more on this topic, see Nancy Sowell's chapter, "The Internal Family System and Adult Health: Changing the Course of Chronic Illness," from the book *Internal Family Systems Therapy: New Dimensions* [2013], which covers various important applications of IFS.)

HELP, NOT BLAME

We are used to our clients reacting strongly when we suggest that a part might have an agenda regarding their physical symptoms. A few factors probably contribute, including polarized parts blaming each other for all that afflicts, a cultural bent toward blaming the victim, and religious or New Age teachings that imply we create our own illnesses. Even so, clients often get curious if we preface the question about parts and illness thoughtfully. I (RS) say something like this:

"Some parts might be influencing your body outside your awareness—and they might be willing to talk to you. This has nothing to do with the notion that you're choosing to be sick. Parts don't ask for permission to affect the body. But they can be too young to know that they're inflicting physical damage. Or they might have some important reason for doing what they're doing. Helping these parts can make a big difference."

If I suspect the client will react negatively to the idea that a part might be creating or exacerbating a physical condition, I simply guide the client to ask for parts who could help heal the body. I have met parts who said they could help if we could convince other parts to relax, and I have met parts who know how to heal the body even when no other part has been involved with harming the body.

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GUIDELINES FOR USING IFS WITH MEDICAL CONDITIONS

IFS therapists use the same basic approach with medical as with nonmedical problems. In the first step we empty our heads of expectations, ask our parts to step back, and embody our Selves. Next,

we ask the client for permission to explore physical illness. Then we proceed with one of several options:

1. We guide the client to focus on a physical symptom, and then we are curious and listen, as we generally do with parts.
2. We speak directly to the symptom as a part.
3. If no parts are involved with the symptom, we guide the client to ask if any part has information about the symptom.
4. We ask the client how he feels toward the symptom, and then we ask parts who hate or fear the symptom if they are influencing the client's body or medical compliance.
5. We guide the client to ask for parts who know how to heal the body, either generally or in this particular area.

These various approaches often help parts to speak up, and if they do, we can proceed to relate to the symptom as we would any part. The guidelines for exploring medical illness are summarized in [Box 5.1](#).

BOX 5.1. Options for Exploring the Physical Symptom as a Part

1. Guide the client to focus on a physical symptom, be curious, and listen.
2. Use direct access to speak directly to the symptom as a part.
3. If no parts are involved with the symptom, guide the client to ask if any part has information about the symptom.
4. Ask the client how he or she feels toward the symptom.
5. Ask if any parts who hate or fear the symptom are influencing the client's body or medical compliance.
6. Guide the client to ask for parts who know how to heal the body, either in general or in this particular area.

EXILES CAUSE PHYSICAL PAIN AND ILLNESS TO GET ATTENTION OR COMMUNICATE AN EXPERIENCE

If we opt to start by treating the symptom as a part, either guiding the client to focus and be curious or using direct access and speaking to the symptom ourselves, we eventually get around to asking

the part why it is affecting the client's body in this way. If it says it wants the client to understand its pain, or that it hasn't been able to get the client's attention any other way, the part is likely to be an exile. In this case, the client's Self asks the part to stop intensifying pain and negotiates with protectors for permission to help the exile.

Here is an example of a panicked exile exacerbating pain. Jonny was an 18-year-old college freshman who took a medical leave due to panic attacks that were causing a big increase in chronic stomach pain, which his doctor had diagnosed as irritable bowel syndrome (IBS). His doctor recommended that Jonny come to therapy because he understood that stress exacerbates IBS. At the time of this session, Jonny had had a couple of IFS therapy sessions and understood the concept of parts.

THERAPIST: Are you in pain now, Jonny?

JONNY: I pretty much always have some pain, but sometimes I feel like I can't breathe and it gets worse all of a sudden.

THERAPIST: Do you have asthma?

JONNY: No. There's nothing wrong with my lungs. It's IBS.

THERAPIST: In my experience, parts can influence our bodies without our awareness. If some part is making it hard for you to breathe, I bet it would have a reason. Can we ask?

[The therapist introduces the idea of talking to parts about his physical symptoms.]

JONNY: It's not like I wanted to drop out of school, you know.

[Some part feels that Jonny is being accused of causing the problem.]

THERAPIST: I know. Parts don't ask for permission before they do things to the body. They have their reasons and we don't control them. Sometimes a part is too young to even understand that it's doing damage. So if a part is causing or exacerbating an illness, all we can do is offer to help.

In my experience, though, parts appreciate that offer, one way or the other.

[The therapist's audience here is the part who feels Jonny is being accused.]

JONNY: All right.

THERAPIST: Shall we focus on the stomach pain or the breathing?

[The therapist invites Jonny to choose where to begin.]

JONNY: The breathing.

THERAPIST: What do you notice?

[Fleshing out the target part.]

JONNY: Tightness. It's hard to breathe.

THERAPIST: How do you feel toward that?

JONNY: I just don't want this body.

[Jonny's Self doesn't answer, a part does.]

THERAPIST: Okay. And given that you don't want this body, how do you feel toward that tightness of breath?

[The therapist repeats the part's detecting question.]

JONNY: I'm scared of it.

THERAPIST: Okay. Can I talk to it directly?

[The therapist quickly switches to direct access, asking permission to speak to the part on the assumption that many reactive parts will be activated around Jonny's physical problems and it will be quicker to speak to the part directly at this point.]

JONNY: Yes.

THERAPIST: So I want to talk to the part who can't breathe. Are you there?

JONNY: Yes.

THERAPIST: What do you want Jonny to know?

JONNY: Help!

THERAPIST: What kind of help do you need?

JONNY: I need to get out of the refrigerator!

THERAPIST: Do you know what that means, Jonny?

JONNY: Yeah. I'm kinda surprised. When I was 6 years old my older brother and his friends put me in an old refrigerator in the basement. My father was planning to take the door off, but the handle was broken anyway, so I'm sure they thought I could get out easily. They were just trying to scare me. But the door got kind of stuck. I managed to scrunch around and kick it open. I never told anyone, not even my brother.

THERAPIST: Ask if the 6-year-old is still in there.

JONNY: He says yes. I had no idea!

THERAPIST: Does he want your help?

[Although this may seem obvious, it's a good idea to keep asking what the part wants.]

JONNY: Yes. But how can I do that?

THERAPIST: You can go back to that time and take care of him. Ask him what he needs you to do that he needed someone to do at the time.

[We call this a do-over. Instead of trying to undo the past, which is a common strategy of protective parts that always fails because it is based on trying to prevent a traumatic experience that has already occurred or deny that it happened, we invite the exile to direct a preferred scenario with a preferred outcome.]

JONNY: He wants me to pull the door off and throw it into outer space.

THERAPIST: Okay, do that.

JONNY: Now I'm taking him out, and he's throwing the whole refrigerator up there and blowing it up with a ray gun.

THERAPIST: Great.

JONNY: And now he wants me to tell his brother that being mean isn't funny.

THERAPIST: *(after a few moments)* Anything else?

JONNY: Yes. He doesn't want me to leave him alone.

THERAPIST: Would he like to leave that time and come live with you in the present?

JONNY: Yes.

THERAPIST: Okay, bring him up to the present. Can he breathe now?

JONNY: There is still this metal band around his chest.

THERAPIST: Does he need your help with that?

JONNY: I clipped it off and he's melting it with the ray gun.

THERAPIST: How's that?

JONNY: Good.

THERAPIST: Was he affecting your gut pain?

JONNY: His fear was making it worse.

THERAPIST: Will that stop now?

[The therapist has turned the session over to Jonny now.]

JONNY: It will help a lot. He can breathe.

THERAPIST: So how is your gut pain right now?

JONNY: Better.

THERAPIST: Where is it on a scale of 1–10?

JONNY: Five.

THERAPIST: Where would it be if it were purely physical right now?

[The therapist gives Jonny this simple tool for rating his physical pain on a numerical scale so he can notice how parts affect his physical condition.]

JONNY: Probably 1–3.

THERAPIST: So what needs to happen to bring it down?

JONNY: I think I just need to recover and be with him for a while.

THERAPIST: So shall we see how it goes?

JONNY: Yeah. Let's see how it goes.

As we hear, Jonny found a 6-year-old who was stuck in a terrifying situation. This part had not created Jonny's IBS, but his panic had been exacerbating Jonny's pain and he did need Jonny's help.

POLARIZED PROTECTORS

In contrast to exiles who want help, if a part says it uses a physical symptom to *do* something to or for the client, like hurt, punish, control, or distract, it is a protector. For example, Greta was a 25-year-old sexual abuse survivor who came to therapy seeking help with her emotional response to having lupus. She had a part (unknown to her) who was amplifying her pain.

THERAPIST: Can I make a suggestion that's a little off the beaten track? Let's ask your body about this illness.

GRETA: Yeah that's strange. I guess it can't hurt.

THERAPIST: Take a moment and listen inside in case there are any objections.

[The therapist double-checks for any parts who might react negatively.]

GRETA: No.

THERAPIST: How do you notice the lupus?

[The therapist guides Greta to find the part in the body.]

GRETA: My joints ache.

THERAPIST: Okay to focus on that?

[The therapist checks again for permission.]

GRETA: Yes.

THERAPIST: Okay to be curious about it?

GRETA: Hmmm. I'm more used to fighting it.

[As explained in [Chapter 3](#), when we are blended with protective parts we lose access to curiosity. Greta's ability to make this observation means she is at least a little [unblended](#) from the polarity between the illness and the parts who fight the illness.]

THERAPIST: Would the parts who fight it be willing to relax for a few minutes and let you be curious?

GRETA: Okay ... now I do feel curious.

THERAPIST: Send your curiosity right into that aching.

GRETA: It says, *I'm saving you*. What the hell does that mean?

THERAPIST: Is it okay to keep being curious?

[Rather than stopping to check on the part who said What the hell does that mean?, the therapist again takes the shortcut of asking if other parts will let her be curious.]

GRETA: Yeah. Let me do that.... Okay. It says, *You'll get hurt*.

[Now Greta hears from a part who is involved with her amplified pain.]

THERAPIST: Do you understand that?

GRETA: I don't know.

THERAPIST: Are you getting any visuals?

[Finding, focusing, and fleshing out protectors is the first part of getting to know them, as we detail in [Chapter 8](#). Here the therapist returns to focusing and fleshing out the target part, who is newly identified.]

GRETA: Yes. It's a like a cloud with a head—a big genie leaning over me.

THERAPIST: Ask the genie how old it thinks you are.

[This question reveals whether a protector has any awareness of the client's present-day life; and, if not, it either reveals the exile or a polarized protector, because that's who the part is seeing.]

GRETA: Five.

THERAPIST: Does that make sense?

[We ask this question often because we want to know if the client understands what parts are trying to communicate. If not, we want to be sure she has the opportunity to ask for clarification. If she does understand, we go on.]

GRETA: Yes, now I understand.

THERAPIST: How do you feel toward the genie now?

GRETA: I'm clarifying this: I'm not 5 years old anymore. He looks suspicious.

THERAPIST: Let him know you could help the 5-year-old, with his permission.

GRETA: He looks alarmed.

THERAPIST: Of course you would need his permission. We're not here to take anything away from him and we don't want him to go away. He could get to know you and decide if you could help.

[This genie is most likely a firefighter, a reactive emergency worker with a strong need to be in control. Firefighters often need reassurance that we are not trying to defeat them and will not try to override them.]

GRETA: He likes that idea.

THERAPIST: Ask him to look you in the eye and let you know who he sees there.

[Mike Elkin, a senior IFS lead trainer, developed this technique. Eye contact, if parts cooperate, can be a very effective way of detecting reactive parts or, conversely, revealing the Self and introducing it to the target part.]

GRETA: He sees a frightened little girl.

THERAPIST: Let the little girl know we're going to arrange things so you can help, and for now put her in a comfortable room. Ask her to wait there till you come back. Is she willing?

[Greta will make no headway with the genie until this exile unblends. Putting a part in a comfortable room—the room technique—helps the target part separate. We specify that the room be comfortable so the part does not return to a room that has been uncomfortable or dangerous. The room technique can be particularly useful for unblending because parts experience our suggestions literally and the system feels reassured when a controversial target part is safely sequestered.]

GRETA: Okay. She's in a playroom.

THERAPIST: Now ask the genie to look you in the eye again and let you know who he sees there.

GRETA: He sees me! He's surprised.

THERAPIST: What has he been doing to the body to protect the little girl?

GRETA: He has some levers: pain, swelling, headaches, heat.

THERAPIST: Would he be willing to stop pulling those levers long enough to let you help the little girl?

GRETA: He looks worried when you say that.

THERAPIST: He must have a good reason for doing all this stuff to the body. But what worries him? What would happen if he stopped?

[The therapist validates the genie's positive intent proactively.]

GRETA: He doesn't want me to go home for the holidays.

[In return, the genie is cooperative and specific.]

THERAPIST: I see. Can you agree to those conditions?

[The therapist does not pause at this point to go into history. Greta and her parts know the background to this concern. Instead, the therapist continues to negotiate an agreement between Greta and the genie.]

GRETA: I should go home. My mother is ill.

THERAPIST: I see. So the genie does not want you to go home, but another part wants you to go home. How does the genie respond?

[The therapist sees and names a polarity.]

GRETA: He puffed up like a storm cloud. He says he'll blow me up. He looks really upset.

THERAPIST: So the genie feels strongly that you should not go home, but the other part believes you should go home to see your mother. Is that right?

[The therapist names the polarity again.]

GRETA: Yes.

THERAPIST: So what needs to happen?

[The therapist hands Greta's Self the baton. She thinks for a minute.]

GRETA: Well, if the genie will let up, I won't go home. I'll FaceTime Mom instead.

THERAPIST: Does he agree?

GRETA: He doesn't object. He just doesn't want me to be around my stepbrother. He agrees for now.

THERAPIST: Seems like that's all we need for now. How do you feel?

GRETA: Like a steam valve opened.

As we see with this interaction, the genie was intensifying Greta's symptoms of lupus to keep her managers from putting her in situations where a 5-year-old part would be retraumatized by a stepbrother, and probably where a rageful part would be activated as well. When Greta made a deal to honor his concerns (rather than letting managers dictate her behavior), he was willing to take the pressure off her body. The next step would be to get his permission (and the permission of those managers) to help the exile.

OTHER PROTECTIVE MOTIVES

A manager may use a symptom to preempt feelings that it believes the client can't handle. It fears that the exile (or an exiled young protector like anger) will overwhelm the client emotionally. For the same reason, a firefighter may use a symptom to distract from feelings. In these cases we do exactly what we would do with a nonmedical problem: We ask the part to let up on the symptom while we negotiate with the exile not to overwhelm. For example, Frances, who was a champion college swimmer, had been forced by back pain to stop training in the buildup to a competition.

THERAPIST: How would you feel about talking to the back pain itself?

FRANCES: Sounds silly.

THERAPIST: Is it okay to do something silly?

[The therapist persists.]

FRANCES: Why not?

THERAPIST: Okay. Focus on the pain. How do you feel toward it?

FRANCES: Well, now you've got me curious!

THERAPIST: Great. What does it want you to know?

FRANCES: I'm mad!

[This is a protector, but one who has been effectively exiled by more diplomatic managers, as we hear below.]

THERAPIST: You are?

FRANCES: *(closing her eyes)* Yes.

THERAPIST: Do you know about this, Frances?

FRANCES: I can't be angry now.

THERAPIST: One part of you is angry and another says it's not okay to be angry?

[The therapist names the polarity.]

FRANCES: I loved my coach. I idealized her. Why do I do that?

THERAPIST: I see. So what does the back pain say to you?

FRANCES: I'd like to kill her.

THERAPIST: What do you notice when you hear that?

[The therapist is interested in how other parts will respond to this anger.]

FRANCES: Oh my god! Are you kidding me?

THERAPIST: Would the part who just spoke up be willing to let you listen to the angry one for a minute as long as it doesn't overwhelm you?

FRANCES: It reluctantly says okay.

THERAPIST: Good. Ask the angry one to separate as well, and not overwhelm so you can listen.

FRANCES: It says I'm always the sucker.

THERAPIST: The angry part thinks you're a sucker?

FRANCES: Yes. I don't pay attention to warnings.

THERAPIST: Warnings?

FRANCES: From the angry part.

THERAPIST: Does that make sense?

[The therapist checks to see if the more conciliatory part is edging back in or if Frances's Self is still available to hear the angry part.]

FRANCES: Yes. It's true. I rarely speak up when I should.

THERAPIST: The angry part is mad at you. Is it hurting your back?

FRANCES: Yes.

THERAPIST: If you helped the part who doesn't let you speak up, would the angry part be willing to stop hurting your back?

FRANCES: It's too late. I want to quit.

[This is not a direct response to the therapist's question.]

THERAPIST: You have a part who wants to quit now. What do you say to it?

[The therapist just follows the client's lead.]

FRANCES: I can tell her how I feel. If she reacts badly, well, then I can quit.

[This response strikes the therapist as being more Self-led.]

THERAPIST: Would the angry part let you speak for it when you talk to your coach?

FRANCES: It's afraid I won't do it.

THERAPIST: I see. If the part who doesn't speak up would let you handle this, would the angry part let you speak for it?

FRANCES: Yes.

THERAPIST: How does that sound to you?

FRANCES: Well, I do know what needs to be said. If I also remember what I feel grateful for, I think I can say it the right way.

THERAPIST: The angry part agrees?

FRANCES: Yes.

THERAPIST: How does your back feel now?

[Frances stands up and moves carefully from side to side.]

FRANCES: It's a bit looser.

As we hear, Frances is in the grip of a polarity between a compliant protector and an angry protector, with the latter causing enough back pain to either force Frances to drop off the team (and in this way move away from her coach) or speak up. Once these polarized parts unblend, Frances is able to set the intention to speak for the angry part with her coach, and the angry part begins to let up on the back pain.

Note that the therapist did not have to use this session to explore what happened between Frances and her coach. If Frances had said she was not yet ready to speak up or needed to think through what she wanted to say, they could have used another session to role-play the upcoming conversation with her coach. But in any case it would be important to return to exploring the constraints (burdens) that caused Frances's compliant manager to stop her from being assertive, and this would point in the direction of an exile.

IF PARTS START IT, THEY CAN OFTEN STOP IT

In general, parts can give the client a headache, stomach pains, muscle clenching, back pain, nausea, exhaustion, the urge to sleep, a pounding heart, chills, numb hands and feet, and much more. They can also send intrusive thoughts and images into the client's consciousness that cause physical responses. But when we ask them to be direct about their wants and needs rather than hurting the client physically or taking him out mentally, and when they believe we will pay attention to their concerns if they stop, dramatic shifts can take place.

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SOME CAVEATS ABOUT IFS AND MEDICAL ILLNESS

Here are a few caveats about using psychotherapy to treat physical ailments. Given the outcomes of IFS research to date (Shadick et al., 2013) along with the experience of many knowledgeable IFS practitioners, we believe that healing exiles and liberating protectors can have a positive effect in a wide variety of medical conditions. Along these lines, we are excited by new findings about the pervasive role of inflammation behind many diseases. Consider this quote from a 2019 article by Jonathan Shaw in *Harvard Magazine*, “Because the idea that inflammation—constant, low-level, immune-system activation—could be at the root of many noncommunicable diseases is a startling claim, it requires *extraordinary* proof. Can seemingly unconnected illnesses of the brain, the vasculature, lungs, liver, and joints really share a deep biological link? Evidence has been mounting that these common chronic conditions—including Alzheimer's, cancer, arthritis, asthma, gout, psoriasis, anemia, Parkinson's disease, multiple sclerosis, diabetes, and depression among them—are indeed triggered by low-grade, long-term inflammation. But it took that large-scale human clinical trial to dispel any lingering doubt: the immune system's inflammatory response is killing people by degrees” (para. 4)

If this proves true and, as we found in the arthritis study, IFS can reduce chronic inflammation, the implications are enormous. At the same time, there are limits. Tissues may be damaged to the point that full recovery is impossible, regardless of how much a client's inner system harmonizes. For example, bulimia does major damage to the teeth and causes some clients to need extensive dental work, which many can ill afford. And while reductions in inflammation certainly helped some of the arthritis patients in our RA study, those who had severely deteriorated joints needed replacements. Along the same lines, once some physiological processes have been set in motion,

they will not respond promptly to a part's transformation and will run their course. Finally, full recovery may require other changes, including to diet, medication, environmental stressors, and medical compliance. And we should never ignore other promising avenues of healing such as meditation, massage, naturopathic health care, and acupuncture.

* * *

CONCLUSION

Parts live in the body, have bodies, are affected by the body, and can affect the body if they wish. The body can be a message board for exiles or a battleground for warring protectors. Burdens can be in or on the body, but can also lurk around it. Burdens crowd and, as the ACE studies document, damage the body. When protectors also damage the body, their motives often reference a time past, and they can be helped to stop.

As the RA study demonstrated, IFS can help with physical illness. Parts can often stop the physical symptoms they initiate. Additionally, some parts can help with healing. Therefore, hope is warranted on this topic. Nevertheless, we recommend a mindful approach with clients. Some will have had the experience of being blamed for their illness by family, friends, or treaters and many will be hearing a lot of internal blame. When medicine is not curious about the inner system and its burdens, would-be healers run the risk of missing the message because they are trying to kill the messenger. In IFS we invite the messenger and explore physical illness the way we always do: by asking.